UTI AND PYELONEPHRITIS IN PREGNANCY: WHAT TO DO NOW

FD-ANT-UTI · Companion to UTI and pyelonephritis in pregnancy · ACOG CC No. 4 · IDSA 2019 · ACOG CO 797 / 717 · SMFM #67 · Labor and delivery / OBED · Decision support only, not unit policy

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PROVISIONAL

HOW TO READ THIS CHART: start by choosing In labor / admitted or OB ED triage. **Doses, regimens and durations live on the companion card; this chart lists none.** Every treatment node says see ANT-UTI card. Pregnant pyelo is always admitted.

SHADING KEY: WHITE = home / ASB or cystitis · LIGHT GRAY = first step / screen · MID GRAY = admit / inpatient pyelo · DARK GRAY = sepsis — SYS-03 first

START: IN LABOR / ADMITTED, OR OB ED TRIAGE?

Choose the path that matches how she presents. Pregnant or postpartum: GYNB-06 is never used.

IN LABOR / ADMITTED · WHITE

Sepsis screen first (CSM tool). Positive → node 5. Fever in labor: IAI (LND-04) first; send UA + culture. Confirmed pyelo → node 4; see ANT-UTI card. GBS bacteriuria any count → IAP. Postpartum inpatient → node 6.

OB ED TRIAGE · WHITE

Not in labor (any GA) or postpartum ≤12 wk. Continue to the OB ED branch below.

1. UNIVERSAL FIRST STEP · LIGHT GRAY

Vitals with sepsis screen (CSM tool). GA/viability; FHT or EFM/toco. Contractions, bleeding, ROM or abnormal FHR → L&D; in parallel. Sepsis screen positive or any red flag → node 5 (SYS-03) now. Otherwise: 2 ASB, 3 cystitis, or 4 pyelo.

2. ASB · WHITE

Uropathogen ≥100,000 CFU/mL: treat, culture-directed — see ANT-UTI card. Vaginal flora only: no treatment. GBS at ANY count: chart flag for IAP; antepartum treatment only if ≥100,000 or symptomatic — see ANT-UTI card. Symptoms → node 3 or 4. Home → node 7.

3. ACUTE CYSTITIS · WHITE

UA + culture before the first dose. Pyuria with negative culture, or vaginal symptoms → NAAT and ANT-STI (first alternate). Treat empirically — see ANT-UTI card. Fever, CVA tenderness or vomiting → node 4. Home if afebrile and tolerating PO → node 7.

4. PYELONEPHRITIS — ALWAYS ADMIT IN PREGNANCY · MID GRAY

Fever ≥38.0 °C + urine findings + flank pain/CVA tenderness (± N/V). No outpatient branch in pregnancy. UA + culture before the first IV dose (do not delay the dose). Start IV antibiotics in OBED — see ANT-UTI card. Measured fluids — ARDS/pulmonary edema risk. Clinically improved → oral step-down and total course — see ANT-UTI card. Not improving at 48–72 h → recheck culture; renal ultrasound; urology / ID as indicated. Septic → node 5.

5. SEPSIS RED FLAGS — SYS-03 FIRST · DARK GRAY

Open SYS-03 maternal sepsis flow first; ANT-UTI is the source card (antibiotic choice, source control). Cultures + lactate, antibiotics ideally within 1 h, crystalloid and pressor per SYS-03. Obstruction/stone/abscess → urology now. Sepsis alone is not an indication for delivery.

6. POSTPARTUM (≤12 WK) · LIGHT GRAY

No ASB screening or treatment postpartum. Cystitis → node 3 (lactation check on the card). Admit pyelo if any admit criterion (sepsis/red flag, unable to take PO, vomiting, immunocompromise, obstruction/stone, failed oral therapy, unreliable follow-up, within the delivery admission or ≤1 wk after cesarean). **House practice (Michael):** well-appearing, no admit criteria — IV dose plus oral therapy with follow-up in 24–48 h; see ANT-UTI card. Pregnant pyelo is still always admitted. GYNB-06 is still not used.

7. DISPOSITION AND RETURN PRECAUTIONS · WHITE

Home (ASB, cystitis, or postpartum per node 6): afebrile, no CVA tenderness, tolerating PO, call-back named, GBS flag if applicable. Admit: any pyelo in pregnancy; sepsis; postpartum admit criterion. Return for fever, flank pain, vomiting, contractions, bleeding, leaking, decreased FM, shortness of breath, or symptoms not better in 48 h. **House practice (Michael):** routine test-of-cure culture after treated ASB or cystitis (ACOG and IDSA do not require it). **House practice (Michael):** routine suppression after a single pyelo — see ANT-UTI card.

PROVISIONAL — pending Michael review and CSM pharmacy check. Not live. Doses, regimens and durations live only on the companion card (UTI and pyelonephritis in pregnancy). Decision support only, not unit policy. ACOG CC No. 4 dose table and 48–72 h window: pending full-text re-check (secondary summary only).